

Text B

Over exposure to ultra violet (UV) light causes 95% of melanoma Skin cancer be prevented through skin protection and early identification.

Skin protection includes:

- seeking shade especially during summer peak hours of 10am-4pm
- wearing clothing that covers back, shoulders, arms and legs
- wearing a broad-brimmed hat
- wearing wrap round sunglasses

Early identification involves checking moles and freckles for changes that fall into 5 categories of ABCDE :

- Asymmetry when one-half of a mole or birthmark does not match the other
- Border irregularity when the edges are irregular, ragged, notched, or blurred
- Colour variation when the colour is not the same all over, but may have differing shades of brown or black, sometimes with patches of red, white, or blue
- Diameter of the mole is larger than 6 mm (about the size of a pencil eraser) or is growing larger
- Evolving in size, shape, colour, elevation, or another trait (such as itching, bleeding or crusting). (This last point is likely the strongest of all of the warning signs)

Melanoma of the skin can develop anywhere on the body including:

- scalp – so check through hair
- ears and nostrils
- underarms
- hands and nail beds
- soles of the feet

Questions 8-14

Answer each of the questions, 8-14, with a word or short phrase from one of the texts. Each answer may include words, numbers or both.

- 8 What are increasingly available to carry out thorough checks for early changes?

Type your answer

- 9 The use of solariums has resulted in the increased exposure to what type of radiation?

Type your answer

- 10 Surveys were carried during summers between 2003-04 and 2010-11 to check what sort of compliance over weekends

Type your answer

- 11 What other areas of the body apart from skin and eye can develop melanoma?

Type your answer

- 12** What other areas of the body apart from skin and eye can develop melanoma?

Type your answer

Part-A

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Specialist mole check centres are increasingly available to carry out a thorough check for early changes and monitor at risk moles and spots. Seeking medical advice if any of the ABCDE signs appear is vital.

Text C

Sunscreen use

Three national surveys during summers between 2003-04 and 2010-11 show that sun protection compliance while outdoors on the weekend during peak UV radiation hours was relatively low.

Table 1 Trends in adolescents' weekend sun protection behaviours and sunburn

Adolescents (12 – 17), n= 2,718	2003-04	2006-07	2010-11	Significant changes in 2010-11 %
Respondents outdoors > 15 minutes	80%	82%	77%	? since 2006-07
Time spent outdoors (minutes)	110	111	112	no change
Hat used	38%	29%	23%	? since 2003-04

11

What other areas of the body apart from skin and eye can develop melanoma?

Type your answer

12

What other areas of the body apart from skin and eye can develop melanoma?

Type your answer

13

What hours should a person seek shade in peak summer hours?

Type your answer

14

Working where will increase the risk of developing skin cancer?

Type your answer

Questions 15-20

Complete each of the sentences, 15-20, with a word or short phrase from one of the texts. Each answer may include words, numbers or both.

Text A

Text B

Text C

Text D

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PART B

Text 4

Memo to staff re: Escalation process for dispute resolution

If, on review, the inpatient team disagrees that a patient requires admission to the respective specialty, the inpatient team shall make the onward referral to the alternate specialty. The senior inpatient MO shall provide the alternate specialty with a clinical handover and advice of the time frame in which the patient must be reviewed.

In the event of a dispute regarding inpatient team acceptance of the patient, a Consultant level case conference shall be convened. The case conference shall include the ED, original inpatient and alternative inpatient Consultants. The three Consultants shall make a timely clinical decision regarding the patient's admission.

If the matter remains unresolved, then the case shall be escalated to the Executive Director of Medical Services (EDMS) for a clinical and time urgent decision.

Part B: QUESTIONS 1-6

Questions 1-6

4

If the inpatient team cannot agree on accepting a patient who has to attend a case conference?

A

The senior MO from the alternative specialty, the ED consultant and the original inpatient team representative

B

The patient, the senior patient medical officer and the Executive director of Medical services

C

The patient's first consultant, a consultant from the ED and a consultant from the alternative speciality that the inpatient team suggests

PART B

QUESTION 1-6

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Go to:

Part B

Part C

Text 1

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PART B

Text 5

Taking a blood pressure reading manually

Wrap the cuff around the patient's arm and use the INDEX line to determine if the patient's arm circumference falls within the RANGE area. Otherwise, choose the appropriate smaller or larger cuff.

Palpate/locate the brachial artery and position the BP cuff so that the ARTERY marker points to the brachial artery. Wrap the BP cuff snugly around the arm. Ensure the cuff is located about 2.5 cm above the antecubital fossa (crease of the arm).

Perform a preliminary palpatory systolic blood pressure. To achieve this, palpate the arm at the antecubital fossa to locate the brachial artery with your non-dominant hand and ensure the pressure valve is closed. Inflate the bladder until no blood is flowing through the artery and no pulse is palpated. The pressure reading on the sphygmomanometer at this stage is an estimate of the maximum pressure required to measure the systolic pressure. Deflate the cuff and rest the arm for one to two minutes to allow the blood to be released and recirculate.

Part B: QUESTIONS 1-6

Questions 1-6

5 How is a preliminary palpatory systolic blood pressure check performed?

A Feeling for a pulse in the crook of the elbow, inflating the sphygmomanometer cuff until the pulse stops and noting the reading after a minute or two

B Feeling for a pulse in the crook of the elbow, inflating the sphygmomanometer cuff until the pulse stops in the non-dominant hand

C Feeling for a pulse in the crook of the elbow, inflating the sphygmomanometer cuff until the pulse stops and noting the reading

PART B

QUESTION 1-6

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PART B

Text 6

GP Notify guidance for GPs

GP Notify is an automated notification system that informs GPs about: patient admission; patient discharge; patient death. When a patient is admitted, discharged or dies, the automated computer system will inform the patients nominated GP by email or fax of the update. GPs will receive an enrolment confirmation the first time such an event occurs. The form needs to be completed and faxed for automated notifications to continue. To receive notification via fax or email: you will receive an enrolment confirmation form the first time an event occurs on one of your patients. Once the form is completed and returned via fax, automated notifications will commence. If your contact details change, notify the GP Notify Coordinator. Please note that GPs are responsible for ensuring their details are correct and ensuring fax machines or computer systems provide adequate privacy after transmission.

Part B: QUESTIONS 1-6

Questions 1-6

6 When a GPs patients are entered into the GP Notify system, the GP

- A Needs to complete and return a confirmation form by fax to confirm they have received the information
- B Gets notification via email or fax, unless it is the first time on of their patients has been entered into the GP notify system
- C Faxes or emails the GP Notify Coordinator to confirm they have received the message

PART B

QUESTION 1-6

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PART C

Text 1

Heart failure patients have weakened hearts, but researchers say an experimental drug used for the first time in humans may repair heart cells and improve heart function. According to the results of a small phase 1 trial, a single intravenous infusion of the drug cimaglermin was safe and, at high doses, improved heart function for at least three months.

"Right now we have many therapies that we use for heart failure, and these patients [in the study] were on all of those therapies and still had significant heart dysfunction," said lead researcher Dr. Daniel Lenihan. He's a professor of medicine and director of Vanderbilt University's heart clinical research program in Nashville.

People with heart failure often take a combination of drugs, Lenihan said. These include medications to lower blood pressure and diuretics to help remove excess fluid that builds up as a result of the heart's labored pumping ability. In addition, some people have implanted defibrillators or pacemakers. Even with all these options, the death rate among these patients is "unacceptably high," Lenihan said.

Heart failure, a condition where the heart can't pump enough blood to meet the body's needs, is among the leading causes of death worldwide. A significant number of heart failure patients don't respond well to current treatments, particularly those patients whose left lower heart chamber, which pumps blood into the arteries, is weak, Lenihan said.

Part C: QUESTIONS 7-22

Questions 7-14

7

According to first paragraph, cimaglermin

- A is safe when used in high doses
- B improves heart function indefinitely
- C is undergoing trials
- D is used in high doses

8

Which of the following statements does Dr. Lenihan make?

- A Patients who received cimaglermin were on many other therapies
- B Patients who received cimaglermin suffered from heart failure
- C Patients received cimaglermin at the Vanderbilt University

Part-B

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Cimagermin acts as a growth factor for the heart, helping it repair itself following injury, Lenihan said. Specifically, it binds to the HER2 and HER4 receptors on the surface of heart cells that are important for cellular repair and survival, he explained.

Researchers have tried using stem cells to repair heart muscle in much the same way, he said, but these efforts have not been effective. "You don't see any sustained effect," he added.

A phase 1 trial like this one is designed to see if a new drug is safe, not to test its effectiveness. Before cimagermin could be used to treat patients, it must prove its worth in a series of progressively larger and challenging trials and then be approved by the U.S. Food and Drug Administration. The process can take several years. Based on these preliminary findings, larger trials are being planned, Lenihan said. "This drug, although still in an experimental phase, might be an important way to improve heart function in patients with heart failure," he said.

For the study, Lenihan and his colleagues randomly assigned 40 patients to get an infusion of cimagermin or a placebo. Compared with patients who received a placebo, patients given a high dose of cimagermin had a sustained increase in the heart's ability to pump blood. The improvement lasted 90 days, with the maximum increase in heart function reached in 28 days, the researchers found. The most common side effects were headache and nausea directly after receiving the drug. One patient who received the highest dose of cimagermin developed abnormal liver function, which cleared up over a two-week period, Lenihan said.

Despite the encouraging results of this first trial, a lot more testing will be needed before cimagermin can be considered a standard treatment for heart failure,

9

Which of the following therapies are not mentioned by Dr. Lenihan?

A

Therapy to decrease high blood pressure

B

Diuretics to get rid of excess fluid

C

Defibrillators and pacemakers

D

Therapy to increase low blood pressure

10

What is heart failure?

A

A condition in which the lower heart chamber is weak

B

A condition in which the left lower chamber pumps blood into the arteries

C

A condition in which the blood pumped by the heart can't satisfy the body's requirements

D

A condition for which there is no treatment

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Despite the encouraging results of this first trial, a lot more testing will be needed before cimaglermin can be considered a standard treatment for heart failure, Bishopric said. "These findings need to be replicated in larger trials, and you have to be able to predict whether improved heart function from cimaglermin will help people live longer and feel better," she noted.

11

According to Dr Lenihan, cimaglermin

- A replaces receptors on the heart's surface.
- B helps to enhance the size of the heart.
- C aids the heart's recovery process.
- D has not been as effective as stem cells.

12

Which of the following considerations is not part of the approval for a new drug?

- ☐ A Safety
- ☐ B Effectiveness
- ☐ C Challenging trials
- ☐ D Planned trials

13

way, he said, but these efforts have not been effective. You don't see any sustained effect," he added.

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13

Which of the following results were achieved with cimaglermin?

- A Cimagermin didn't produce any results
- B The effects of cimagermin were adverse
- C Cimagermin improved the heart's ability to pump blood
- D Cimagermin produced several side-effects

14

Which of the following is the most suitable title for the article?

- ☐ A A promising new drug treats heart failure
- ☐ B A new heart failure drug shows promise in first human trial
- ☐ C A new heart failure drug shows promise when tested on animals
- ☐ D A promising new drug that treats heart failure has a bright future ahead

PART C

Text 2

After years of studies that seemed to swing between dire warnings and cheery promises about what our favourite caffeinated beverages do and don't do, much of the recent science regarding coffee and tea is generally positive.

The WHO's International Agency for Research on Cancer recently took coffee off its list of suspected carcinogens, and some research suggests it could help keep colon cancer from coming back after treatment. Other studies suggest drinking coffee might stave off Alzheimer's and Parkinson's diseases.

Various studies have pointed to tea drinkers having lower odds of skin, breast, and prostate cancers. Researchers are still trying to pinpoint the exact ways that happens. But tea, particularly green tea, is rich in compounds like antioxidants, which can limit cell damage and boost the immune system; and polyphenols, which have been shown to lower blood pressure and cholesterol. It also may help stave off Alzheimer's disease through a polyphenol known as EGCG, which prevents the formation of plaques that are linked to that brain-damaging illness.

Is one better for you than the other? Experts say that's hard to say. That's because it's difficult to separate out their different ingredients, their role in your diet, and their effects on different body systems. "I think people are looking at both coffee and tea and how they affect everything, including cancer and GI disease and cardiovascular diseases," says Elliott Miller, MD, a critical care medicine specialist at the National

21

What does "linear" suggest in seventh paragraph?

- A That the more coffee people drank the lower the risk of colon cancer was
- B That coffee intake had no significant impact on colon cancer
- C That drinking coffee is proportional to the reoccurrence of colon cancer
- D That drinking coffee is proportional to the prevention of the reoccurrence of colon cancer

22

Which of the following points is not made in eighth paragraph?

- A Coffee can affect people differently.
- B People's lifestyles may play a part in whether they drink tea or coffee.
- C There is no connection between genetics and whether people drink tea or coffee.
- D Research has shown that there is a direct connection between drinking coffee and smoking

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Is one better for you than the other? Experts say that's hard to say. That's because it's difficult to separate out their different ingredients, their role in your diet, and their effects on different body systems. "I think people are looking at both coffee and tea and how they affect everything, including cancer and GI disease and cardiovascular diseases," says Elliott Miller, MD, a critical care medicine specialist at the National Institutes of Health.

Miller and his colleagues recently looked at signs of heart disease in more than 6,800 people from different backgrounds across the country. About 75% drank coffee, while about 40% reported drinking tea. Drinking more than one cup of tea regularly was linked to less buildup of calcium in arteries that supply blood to the heart, a development that can lead to heart disease. Coffee didn't have an effect either way on heart disease, but that was significant in itself, Miller says. "Very often patients will ask their doctors, 'Hey, doc, I've got coronary artery disease, or I've got risk factors like high blood pressure or cholesterol. Is it safe for me to drink coffee?' Because everyone thinks drinking coffee makes your heart excited and is potentially bad," Miller says. "So finding that it's neutral, I think, is pretty important."

Researchers say it's hard to pinpoint exactly how both drinks affect health. Both coffee and tea are "complex beverages" that contain a variety of ingredients. They include caffeine, polyphenols, and antioxidants -- compounds researchers are studying for their potential cancer-fighting properties, says Lisa Cimperman, a clinical dietitian at University Hospitals Case Medical Center. "It's more of a dynamic interaction than one single compound," Cimperman says. Some people have tried to isolate one element in tea or coffee that they think is the secret to one effect or another, "and then they realize that it doesn't have the same effect."

Questions 15-22

15

What did past studies suggest about coffee and tea?

- A They produced clear evidence that they were harmful.
- B They produced clear evidence that they were beneficial
- C They produced evidence that they could be either harmful or useful.
- D They didn't produce clear evidence on whether they are harmful or useful.

16

According to some research:

- A Coffee can be used as an effective treatment for Alzheimer's and Parkinson's diseases.
- B Alzheimer's and Parkinson's diseases can be caused by coffee.
- C Alzheimer's and Parkinson's diseases can effectively be prevented by coffee.
- D Drinking coffee may help with the prevention of Alzheimer's and Parkinson's diseases.

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Researchers say it's hard to pinpoint exactly how both drinks affect health. Both coffee and tea are "complex beverages" that contain a variety of ingredients. They include caffeine, polyphenols, and antioxidants -- compounds researchers are studying for their potential cancer-fighting properties, says Lisa Cimperman, a clinical dietitian at University Hospitals Case Medical Center. "It's more of a dynamic interaction than one single compound," Cimperman says. Some people have tried to isolate one element in tea or coffee that they think is the secret to one effect or another, "and then they realize that it doesn't have the same effect." Cimperman said drinking tea has been linked to lower risks of cancer and heart disease, improved weight loss, and a stronger immune system. Meanwhile, studies point to coffee as a potential way to head off not just Parkinson's but type 2 diabetes, liver disease, and heart problems, Cimperman says.

Another recent study, led by Charles Fuchs, MD, director of the Gastrointestinal Cancer Center at Boston's Dana-Farber Cancer Institute, found regular coffee drinking may help prevent colon cancer from coming back after treatment. In his study of nearly 1,000 patients, Fuchs says, there was a "significant and linear" association between drinking coffee and lower risk of colon cancer returning in those who drank four or more cups a day. "The more coffee they drank, the lower risk of recurrence." But the researchers aren't clear on which element of the drink contributed to that result, and there didn't seem to be any effect from drinking tea, he says. "I think you can have two or more cups a day without any concern, and certainly that may benefit you," Fuchs says. But what about for those who don't drink coffee? "If it was somebody who hates the stuff and asks, 'Should I drink it?' I'd say no. I'd counsel them about diet and exercise and avoiding obesity as measures I think would have a

17

Which of these effects is not attributed to drinking tea?

A

Decreasing the risk of breast cancer.

B

Increasing blood pressure.

C

Strengthening the immune system.

D

Decreasing the risk of developing Alzheimer's disease

18

What does fourth paragraph suggest?

A

That it's easy to determine the positive effects of coffee and tea

B

That it's not easy to determine the positive effects of coffee and tea.

C

That it's not easy to determine which one is healthier: coffee or tea.

D

That the positive effects of coffee and tea are determined by their role in our diet

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Other researchers are asking questions about what role genetics and lifestyle play into the effects of drinking coffee or tea. For instance, coffee and cigarettes once went together like ... well, like coffee and cigarettes, which cause cancer and heart disease. Some people's bodies process coffee differently than others, says Martha Gulati, MD, head of cardiology at the University of Arizona College of Medicine in Phoenix. Meanwhile, a preference for tea over coffee might reflect other healthier behaviors, she says. "Does someone who drinks tea do yoga or meditation more?" Gulati says. "I'm not necessarily saying they're associated, but do they exercise more? Are they drinking things like green tea to maintain their weight better than other types of drinks?"

19

What is Dr. Miller's opinion on the findings?

A

 Coffee increases the risk of heart disease in 75% of the people who took part in the research.

B

 Coffee has no effect on heart disease.

C

 The fact that coffee has no effect on heart disease is noteworthy.

D

 From now on, patients will stop asking doctors if coffee is safe to drink.

20

What makes coffee and tea beneficial, according to Cimperman?

A

 A secret ingredient found in coffee and tea.

B

 The relations among the complex ingredients coffee and tea contain

C

 One single compound isolated by some scientists

D

 The fact that they help to fight many different diseases

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Another recent study, led by Charles Fuchs, MD, director of the Gastrointestinal Cancer Center at Boston's Dana-Farber Cancer Institute, found regular coffee drinking may help prevent colon cancer from coming back after treatment. In his study of nearly 1,000 patients, Fuchs says, there was a "significant and linear" association between drinking coffee and lower risk of colon cancer returning in those who drank four or more cups a day. "The more coffee they drank, the lower risk of recurrence." But the researchers aren't clear on which element of the drink contributed to that result, and there didn't seem to be any effect from drinking tea, he says. "I think you can have two or more cups a day without any concern, and certainly that may benefit you," Fuchs says. But what about for those who don't drink coffee? "If it was somebody who hates the stuff and asks, 'Should I drink it?' I'd say no. I'd counsel them about diet and exercise and avoiding obesity as measures I think would have a similar benefit."

Other researchers are asking questions about what role genetics and lifestyle play into the effects of drinking coffee or tea. For instance, coffee and cigarettes once went together like ... well, like coffee and cigarettes, which cause cancer and heart disease. Some people's bodies process coffee differently than others, says Martha Gulati, MD, head of cardiology at the University of Arizona College of Medicine in Phoenix. Meanwhile, a preference for tea over coffee might reflect other healthier behaviors, she says. "Does someone who drinks tea do yoga or meditation more?" Gulati says. "I'm not necessarily saying they're associated, but do they exercise more? Are they drinking things like green tea to maintain their weight better than other types of drinks?"

21

What does "linear" suggest in seventh paragraph?

A

That the more coffee people drank the lower the risk of colon cancer was

B

That coffee intake had no significant impact on colon cancer

C

That drinking coffee is proportional to the reoccurrence of colon cancer

D

That drinking coffee is proportional to the prevention of the reoccurrence of colon cancer

22

Which of the following points is not made in eighth paragraph?

A

Coffee can affect people differently.

B

People's lifestyles may play a part in whether they drink tea or coffee.

C

There is no connection between genetics and whether people drink tea or coffee.

D

Research has shown that there is a direct connection between drinking coffee and smoking

Go to:

Part B

Part C

Text 1

Text 2

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Increasingly, younger people are having TKR as it improves their quality of life and their ability to carry on working. However, this also increases the likelihood that parts of the artificial joint will wear out or the joint will loosen. This means further surgery may be required and a revision of a TKR is a more complex procedure and the patient may not be in good health.

Text B

Clinical assessment

- Complete a thorough medical history check to ensure fitness for anaesthetic and rehabilitation
- Knee x-rays
- Physiotherapy assessment of mobility and gait
- Check for symptoms of:
 1. severe pain, swelling and stiffness in knee joint
 2. mobility is reduced
 3. knee 'giving way'
 4. knee locking or clicking
 5. inability to straighten the knee
 6. interference with quality of life and sleep
 7. difficulty with, or unable to carry out everyday tasks, such as shopping or getting out of the bath
 8. feeling depressed because of the pain and lack of mobility

Questions 8-14

Answer each of the questions, 8-14, with a word or short phrase from one of the texts. Each answer may include words, numbers or both.

- 8 What type of analgesia is administered using a patient controlled analgesia system (PCAS)?

Type your answer

- 9 What do the initials PKR stand for?

Type your answer

- 10** What are the most common complications following knee replacement surgery?

Type your answer

- 11** During the clinical assessment, what does a Physiotherapist assess?

Type your answer

Part-A

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Text B

Clinical assessment

- Complete a thorough medical history check to ensure fitness for anaesthetic and rehabilitation
- Knee x-rays
- Physiotherapy assessment of mobility and gait
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 - severe pain, swelling and stiffness in knee joint
 - mobility is reduced
 - knee 'giving way'
 - knee locking or clicking
 - inability to straighten the knee
 - interference with quality of life and sleep
 - difficulty with, or unable to carry out everyday tasks, such as shopping or getting out of the bath
 - feeling depressed because of the pain and lack of mobility
 - unable work or have a normal social life

Text C

Potential complications

11

During the clinical assessment, what does a Physiotherapist assess?

Type your answer

12

What is the main reason a TKR is carried out for?

Type your answer

13

What is the minimum knee bend a patient should have on discharge from hospital?

Type your answer

14

During the clinical assessment, a patient's medical history is checked to ensure they are fit for what?

Type your answer

Questions 15-20

Complete each of the sentences 15-20 with a word or short phrase from one of the

Text A

Text B

Text C

Text D

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Text C

Potential complications

Infection and blood clots are the most common complications following knee replacement surgery.

Infection of the wound which is usually treated with antibiotics, but occasionally the wound can become deeply infected and require further surgery; in rare cases it may require replacement of the artificial knee joint.

The operated leg will be bruised but patients are advised to seek medical help if they find the leg is becoming hot, reddened, hard or painful. Also patients are advised to seek help if they experience chest pains as this may indicate a pulmonary embolism.

Other potential complications include:

- unexpected bleeding into the knee joint
- ligament, artery or nerve damage in the area around the knee joint
- fracture in the bone around the artificial joint during or after surgery
- excess bone forming around the artificial knee joint and restricting movement of the knee
- excess scar tissue forming and restricting movement of the knee
- the kneecap becoming dislocated
- numbness in the area around the wound scar
- allergic reaction to the bone cement

Questions 15-20

Complete each of the sentences, 15-20, with a word or short phrase from one of the texts. Each answer may include words, numbers or both.

15 Deep veined thrombosis prevention options include use of an
Type your answer pump.

16 A person may report knee Type your answer or clicking during the clinical assessment

17 The parts of an artificial knee joint may be held in place with
Type your answer or new bone growth.

18 On discharge, most patients are able to walk Type your answer and go up and down stairs with a walking aid.

19 Type your answer tissue forming and restricting movement of the knee is a potential complication following knee replacement surgery.

20 Type your answer and lifestyle changes are the first choices for managing the knee symptoms

PART A
QUESTION 1-20

Text A Text B Text C Text D

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PART B

Text 1

Risks associated with rubella infection in pregnancy

Maternal rubella infection can result in spontaneous miscarriage, fetal infection, stillbirth, or fetal growth restriction (Reef et al 2000). Congenital infection is most likely if the maternal infection occurs in the first 16 weeks of pregnancy, with congenital rubella syndrome occurring in all fetuses infected before the 11th week and in 35% of those infected at 13–16 weeks (Miller et al 1982). If infection occurs after 16 weeks of pregnancy, the risk of fetal damage is negligible. Features of congenital rubella syndrome include cardiac defects, deafness, ocular defects, thrombocytopenic purpura, haemolytic anaemia, enlarged liver and spleen, and inflammation of the meninges and brain (Sanchez et al 2010). Pneumonitis, diabetes, thyroid dysfunction and progressive panencephalitis are other late expressions of the syndrome (Weil et al 1975; Cooper et al 1995).

Part B: QUESTIONS 1-6

Questions 1-6

1 If a baby is born with visual problems, what stage of pregnancy was it most likely to have been infected with rubella?

A As all babies infected with rubella in their first 11 weeks develop congenital rubella syndrome, the first trimester is when the infection most likely occurred.

B As over a third of all foetuses infected with rubella in weeks 13-16 develop congenital rubella syndrome, the second trimester is when the infection most likely occurred.

C As all babies infected with rubella after their first 16 weeks do not develop congenital rubella syndrome, the third trimester is when the infection most likely occurred.

PART B

QUESTION 1-6

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Go to:

Part B

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Text 1

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PART B

Text 2

Memo to staff re: presence of latex in Flud trivalent influenza vaccine

During the 2018 influenza season, natural rubber latex is present in the sheath covering the needle of trivalent influenza vaccine Flud. Flud is available for people aged 65 years and over through the National immunisation Program (NIP). While reactions to latex are rare, anyone who has a severe allergy to latex should not receive Flud. Patients aged 65 years and over can still be safely vaccinated with the alternative trivalent influenza vaccine, Fluzone High-Dose through the NIP.

Please note there are no safety concerns regarding the Flud vaccine itself.

The sponsor of Flud, Seqirus, commits to update the Product Information and Consumer Medicines Information for this vaccine accordingly.

Action for vaccination providers:

If you are vaccinating patients with Flud, please be aware of this issue and advise patients accordingly. Reassure patients that reactions to latex are rare.

Before administering Flud, confirm with patients that they are not allergic to latex.

Anyone who has a severe allergy to latex should not receive Flud. Patients aged 65 years and over can still be safely vaccinated with the alternative trivalent influenza vaccine, Fluzone High Dose, available through the NIP.

Part B: QUESTIONS 1-6

Questions 1-6

2 The statement below highlights that

A latex is being found in the Flud vaccine itself

B the Flud vaccine itself contains latex

C latex is not being found in the Flud vaccine itself

PART B

QUESTION 1-6

1

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Text 3

Outpatient costs for dressings, home nursing visits and outpatient appointments are key cost drivers for DFU. If spray-on skin is effective, large cost savings to WA Health will be realised immediately through a shortened time to healing, and through a higher proportion of patients achieving complete healing. Any economic benefits are likely to be amplified across Australia and other similar demographic settings where aging populations with increased diabetes rates are considered major future challenges.

Questions 1-6

C see if patients getting care in outpatient departments heal quicker than those having care provided in their own home

PART B

Text 4

Injection technique

As the amount of subcutaneous fat varies between patients, individual patient assessment is vital before carrying out the procedure. It is important to avoid inadvertently injecting the drug into muscle, as intramuscular injection can affect drug absorption; for example, inadvertent administration of insulin into the muscle can lead to accelerated insulin absorption and lead to hypoglycaemia.

A lifted skinfold technique (pinching or bunching the skin) can be used to lift the subcutaneous layer away from the underlying muscle. This method reduces the risk of inadvertent intramuscular injection when undertaken correctly; however, releasing the skin too quickly before the injection is completed or lifting it incorrectly can increase that risk.

Needles need to be long enough to inject the drug into the subcutaneous tissue. They come in lengths of 5cm, 6cm and 8cm. It is suggested the required needle length can be estimated by pinching the skin using the lifted skinfold technique and selecting a needle that is 1.5 times the width of the skinfold.

Part B: QUESTIONS 1-6

Questions 1-6

- 4 What does this information on injection technique tell us about the lifted skinfold technique
- A the lifted skinfold technique allows for a suitable needle choice to be made
 - B the lifted skinfold technique ensures medication to be administered into the muscle layer
 - C the lifted skinfold technique involves releasing the skin quickly to reduce the risk of bruising

PART B

QUESTION 1-6

1 2 3 4 5 6

Go to: **Part B** **Part C** **Text 1** **Text 2**   **00 : 44 : 37**   **Restart** **Pause** **Resume** **Submit**

PART C

Text 1

Surgery that preserves the lung, when combined with other therapies, appears to extend the lives of people with a subtype of the rare and deadly cancer mesothelioma, a new study suggests. Tracking 73 patients with advanced malignant pleural mesothelioma -- which affects the lungs' protective lining in the chest cavity -- researchers found that those treated with lung-sparing surgery had an average survival of nearly three years. A subset of those patients survived longer than seven years. Mesothelioma patients treated with chemotherapy alone, which is standard care, live an average of 12 to 18 months, the researchers said.

Study participants received lung-sparing surgeries and another treatment called photodynamic therapy that uses light to kill cancer cells. Ninety-two percent of the group also received chemotherapy. The study volunteers achieved far longer survival times, said study author Dr. Joseph Friedberg. "When you take the [entire] lung out, it's a significant compromise in quality of life," said Friedberg. He's director of the University of Maryland Medical Center's Mesothelioma and Thoracic Oncology Treatment and Research Center in Baltimore.

"For all intents and purposes, this [lung-sparing surgical approach] is the largest palliative operation known to man, since chances of curing mesothelioma are vanishingly small," said Friedberg. He completed the research while at his previous post at the University of Pennsylvania. "Plus, most of these patients are elderly, so

Part C: QUESTIONS 7-22

Questions 7-14

- 7 According to the information in first Paragraph**
- A Mesothelioma patients have benefited from chemotherapy alone
 - B Mesothelioma patients have benefited from lung-sparing surgery alone
 - C Mesothelioma patients have benefited most from lung-sparing surgery and additional therapies
 - D Only a subset of mesothelioma patients can survive
- 8 Why is lung-sparing surgery better than removing the entire lung?**
- A Because patients survive longer
 - B Because patients have a better life
 - C Because patients' lives are compromised
 - D Because cancer cells are completely eradicated

vanishingly small," said Friedberg. He completed the research while at his previous post at the University of Pennsylvania. "Plus, most of these patients are elderly, so preserving quality of life was really the goal," he added.

About 3,000 Americans are diagnosed with mesothelioma each year, the American Cancer Society says. Many of these people were exposed to the mineral asbestos in industrial occupations, according to the U.S. National Cancer Institute (NCI). Used in products such as insulation, building shingles and flooring, asbestos dust fibers can be inhaled or swallowed, settling in the lungs, stomach or other body areas. Often, it takes decades after exposure for mesothelioma to develop, the NCI says.

Friedberg and his team performed the lung-sparing surgeries on study participants between 2005 and 2013. Overall average survival was 35 months, the study showed. But survival time more than doubled to 7.3 years for 19 patients whose cancer had not spread to their lymph nodes. Most of the patients in the study had stage 3 or stage 4 cancer. Typically, Friedberg said, only about 15 to 20 percent of mesothelioma patients are treated with surgery, which often removes an entire lung as well as the diaphragm and the sac surrounding the heart.

Friedberg said that between 20 and 40 percent of pleural mesothelioma patients with the epithelial subtype might be eligible for lung-sparing surgery. He explained that this surgery removes all visible traces of cancer. It typically has fewer complications and a lower risk of dying in the 90 days following the 10- to 14-hour procedure.

"It's still relatively new that people do lung-sparing surgery for this disease, and it's not established that this is what we need to do," said Friedberg. "I would say this is one of the most lethal cancers known to man. There's a pressing need for new and

9 What does “palliative” refer to in third Paragraph

- A Soothing
- B Life-saving
- C Routine
- D Delicate

10 Which of the following are involve the use of asbestos?

- A Insulation
- B Dust fibers
- C Building shingles
- D Flooring

11 The patients whose cancer had not spread to their lymph nodes

- A Were eligible for the lung-sparing surgery

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"It's still relatively new that people do lung-sparing surgery for this disease, and it's not established that this is what we need to do," said Friedberg. "I would say this is one of the most lethal cancers known to man. There's a pressing need for new and innovative treatments," he noted.

Dr. Daniel Petro, a medical oncologist/hematologist at the University of Pittsburgh Medical Center, said lung-sparing surgery for mesothelioma is also done at academic centers such as his, and he was not surprised by the study's results.

"This [surgical approach] is a step forward with this particular terrible cancer," Petro said, "and we've got to keep coming up with better options to eradicate it."

13 In sixth paragraph, Friedberg draws the reader's attention to

- A All the positive effects of lung-sparing surgery.
- B The fact that mesothelioma is a deadly cancer.
- C The fact that there is a new and highly effective treatment.
- D The necessity of implementing ground-breaking treatments

14 Dr. Daniel Petro believes that the study

- A Is a step forward towards the discovery of a new treatment.
- B Confirms his previous research.
- C Produced results he would have expected
- D Will contribute to eradicating mesothelioma.

PART C

QUESTION 7-22

Go to: [Part B](#) [Part C](#) [Text 1](#) [Text 2](#)

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PART C

Text 2

People with the reading disability dyslexia may have brain differences that are surprisingly wide-ranging, a new study suggests. Using specialized brain imaging, scientists found that adults and children with dyslexia showed less ability to "adapt" to sensory information compared to people without the disorder. And the differences were seen not only in the brain's response to written words, which would be expected. People with dyslexia also showed less adaptability in response to pictures of faces and objects. That suggests they have "deficits" that are more general, across the whole brain, said study lead author Tyler Perrachione. He's an assistant professor of speech, hearing and language sciences at Boston University. The findings, published in the Dec. 21 issue of the journal Neuron, offer clues to the root causes of dyslexia.

Other studies have found that people with dyslexia show differences in the brain's structure and function. "But it hasn't been clear whether those differences are a cause or consequence of dyslexia," Perrachione explained. The chicken-and-egg question is tricky, because years of reading, or years of reading disability, affect brain development. Perrachione said his team thinks it has discovered a cause of dyslexia -- partly because the reduced adaptation was seen in young kids, and not only adults.

A researcher who was not involved in the study called it "groundbreaking." "Frankly,

Part C: QUESTIONS 7-22

Questions 15-22

15 The author describes dyslexic people's brain differences as

- A More varied than many people realize
- B Easier to understand than people expect
- C having wide-reaching effects
- D Absolute

16 What new information did the study reveal?

- A That their brains are more plastic.
- B That their ability to recognize faces and objects is underdeveloped.
- C It confirmed that they have problems recognizing written words.
- D That their brains are less flexible in several ways not considered previously

Go to: **Part B** **Part C** **Text 1** **Text 2**

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A researcher who was not involved in the study called it "groundbreaking." "Frankly, researchers have struggled with understanding the brain bases of dyslexia," said Guinevere Eden, director of the Center for the Study of Learning at Georgetown University Medical Center in Washington, D.C. Scientists have known that brain structure and function look different in people with dyslexia, Eden said, but they haven't known why. "This study makes an important step in that direction," she said. "It gets to the true characteristics of the properties of the neurons [cells] in these brain regions, not just their outward appearance."

People with dyslexia have consistent problems with language skills, especially reading. According to the International Dyslexia Association, as much as 15 percent to 20 percent of the population has symptoms of dyslexia -- including "slow" reading, poor spelling and writing skills, and problems deciphering words that are similar to each other.

The new study aimed to see whether "neural adaptation" might play a role. Adaptation is how the brain improves its efficiency. Perrachione offered an example: When you speak to someone for the first time, the brain needs a little time to get used to that person's voice, speaking rhythms and pronunciation of words, for instance.

But then the brain adapts and stops working so hard to process the other person's speech. In people with dyslexia, however, that adaptation seems to be hindered. "Their brains are working harder to process these sensory inputs," Perrachione said.

The new findings are based on functional MRI scans of adults and children with and without dyslexia. The scans were used to capture the study participants' brain activity

17 What is implied by the chicken-and-egg question in Paragraph 2?

- A The impossibility of discovering the true cause of dyslexia.
- B The fact that many studies constantly reveal various findings about the differences in the brain structure
- C The fact that it is impossible to identify whether alternations are the cause or consequence of dyslexia.
- D The fact that the brain's structure and function are both the cause and consequence of dyslexia.

18 Why has the study been called “groundbreaking”?

- A Because previously scientists hadn't been able to understand the brain bases of dyslexia.
- B Because so far scientists couldn't understand as before why dyslexic brains are different.
- C Because it reveals the brain basis of dyslexia.
- D Because it makes progress in understanding the basis of dyslexia in the human brain

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The new findings are based on functional MRI scans of adults and children with and without dyslexia. The scans were used to capture the study participants' brain activity as they performed a series of tasks. In one experiment, the participants listened to a series of words, read either by a single speaker or several different ones. Overall, the researchers found, people without dyslexia adapted to the single voice, but not to multiple speakers. In contrast, people with dyslexia showed much less adaptation in their brain activity, even when listening to a single speaker. The same pattern was seen when study participants viewed written words.

But the differences went beyond words: People with dyslexia showed less brain adaptation in response to images of faces and objects. That's "surprising," Eden said, since the disorder does not involve apparent problems with recognizing faces or objects. Perrachione speculated on a reason for the findings: The reduced brain adaptation may only "show up" when it comes to reading, because reading is such a complex skill. The brain has no dedicated "reading" area. "Reading is a tool, or technology, that we've invented," Perrachione pointed out. Learning to use that technology requires a complex orchestration of different brain "domains," he explained. And yet, because everyone is expected to read, most people probably do not realize what an accomplishment it is, Perrachione said. Eden agreed. "Learning to read is an astonishing feat and one that we often take for granted," she said.

19 What is the main problem with people with dyslexia according to Perrachione?

A There is no neural adaptation.

B Their brains take much longer to adapt.

C Adaptation occurs instantly.

D There is no sensory adaptation

20 What does the experiment in seventh paragraph reveal?

A That people with dyslexia experience difficulties when listening to multiple speakers.

B That it was difficult for both people with and without dyslexia to adapt to multiple speakers.

C That people with dyslexia have difficulty processing listening as well as written words.

D That brains of people with dyslexia have difficulty adapting to listening as well as reading.

21 When is dyslexia most apparent?

Go to: **Part B** **Part C** **Text 1** **Text 2**

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reading.

21 When is dyslexia most apparent?

A During the process of reading.

B In face recognition.

C In face and object recognition.

D When reading is used as a tool or technology

22 Which of the following is not true about reading?

A It is a very complex skill.

B It can be located in one centre of the brain

C It requires communication among different areas in the brain.

D It is an amazing achievement

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TEXT C

Medication options

- Selective serotonin reuptake inhibitors (SSRIs) help increase the level of a natural "good mood" chemical in the brain called serotonin.
- Tricyclic antidepressants (TCAs) work by raising the levels of the chemicals serotonin and noradrenaline in the brain to help lift the mood.
- Serotonin-noradrenaline reuptake inhibitors (SNRIs) change the levels of serotonin and noradrenaline in your brain
- Lithium carbonate or lithium citrate can be used if all other medication options have not worked

Medication withdrawal symptoms

- Medications should not be stopped suddenly unless there are bad side effects
- Medications should be reduced gradually. This is to prevent withdrawal symptoms that include:

1. an upset stomach
2. flu-like symptoms
3. anxiety
4. dizziness
5. vivid dreams at night
6. sensations in the body that feel like electric shocks

Questions 15-20

Complete each of the sentences, 15-20, with a word or short phrase from one of the texts. Each answer may include words, numbers or both.

- 16 Difficulty Type your answer and increased Type your answer intake are possible physical signs that someone is suffering from depression.
- 17 A person with extreme signs of depression may need admitting Type your answer for treatment.
- 18 A person may be clinically depressed if they are feeling sad all the time for Type your answer weeks or longer.
- 19 Psychodynamic psychotherapy helps a person be more sensitive to hidden Type your answer and how they make them ill.
- 20 Tricyclic antidepressants (TCAs) increase Type your answer and Type your answer to help the person's frame of mind improve.
- 21 During a clinical assessment the doctor conducts a thorough review of the persons Type your answer

PART A

QUESTION 1-20

Non-medication options

- A talking therapy is very helpful for most people with clinical depression.
1. Cognitive behavioural therapy (CBT) helps people understand their thoughts and behaviour. It teaches people how to stop negative thoughts. It can be done online or sitting with a therapist.
 2. Interpersonal therapy (IPT) focuses relationships with others and on problems the person may be having, such as coping with illness or not getting on with a family member.
 3. Psychodynamic or psychoanalytic psychotherapy encourages a person to say whatever is going through your mind. This helps them become aware of hidden meanings or patterns and the effect they have on the person's well being.
 4. Counselling is a form of therapy that helps a person think about their problems and come up with new ways of dealing with them.
 5. Mindfulness involves paying closer attention to the present moment, and focusing on thoughts, feelings, bodily sensations and the world around to improve a person's mental wellbeing.

Electroconvulsive therapy (ECT) is a very controversial, invasive type of brain stimulation that's sometimes recommended for severe depression if all other treatment options have failed, or when the situation is thought to be life-threatening. It is always carried out in a specialist hospital unit by trained medical staff.

Questions 15-20

Complete each of the sentences, 15-20, with a word or short phrase from one of the texts. Each answer may include words, numbers or both.

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PART A

QUESTION 1-20

Text A

Text B

Text C

Text D

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Pacemakers: FDA Safety Communication for Health Professionals

The FDA has reviewed information concerning potential cybersecurity vulnerabilities associated with St. Jude Medical's RF-enabled implantable cardiac pacemakers and has confirmed that these vulnerabilities, if exploited, could allow an unauthorized user to access a patient's device using commercially available equipment.

The FDA and Abbott do NOT recommend prophylactic removal and replacement of affected devices.

Discuss the risks and benefits of the cybersecurity vulnerabilities and associated firmware update with your patients at the next regularly scheduled visit. As part of this discussion, it is important to consider each patient's circumstances, such as pacemaker dependence, age of the device, and patient preference, and provide them with Abbott's Patient Guide.

For pacing dependent patients, consider performing the cybersecurity firmware update in a facility where temporary pacing and pacemaker generator can be readily provided.

Print or digitally store the programmed device settings and the diagnostic data in case of loss during the update.

After the update, confirm that the device maintains its functionality, is not in backup mode, and that the programmed parameters have not changed.

The firmware update process is described in Abbott's Dear Doctor Letter issued on August 28, 2017.

Part B: QUESTIONS 1-6

Questions 1-6

1

What course of action must a healthcare professional take after reading this device recall notice?

A

The health professional should contact their patient so they can discuss issues raised in the recall notice

B

The health professional should discuss the pros and cons resulting from the firmware update at their next routine appointment

C

The health professional should discuss the possible effects of the firmware update and schedule replacing the device as soon as is convenient.

PART B

QUESTION 1-6

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Go to:

Part B

Part C

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PART B

Text 4

Planning for subsequent antenatal visits

At all visits, opportunities should be provided for the woman to share her expectations and experiences as well as discuss any issues and/or concerns that may have arisen since her last visit, including psychosocial support and mental health issues. Women should also be offered information on aspects of health in pregnancy and early parenthood (e.g. nutrition, alcohol, smoking, symptom relief if conditions common in pregnancy are being experienced, breastfeeding, reducing the risk of sudden and unexpected death in infancy [SUDI]). A woman's confidence in her ability to labour, give birth and look after her new baby should be supported throughout antenatal care and antenatal education should also support her in preparing for changes to her life and her relationship with her partner and understanding the physical and emotional needs of the baby. The woman's needs should dictate the type of information and support provided (e.g. while many women will benefit from written information, other forms of information such as audio or video are sometimes more suitable). The woman should also direct the type of issues and questions discussed.

Part B: QUESTIONS 1-6

Questions 1-6

4 What are the main topics women should be given information on during their ante natal period?

A Managing her expectations, knowing which babies are at risk of SUDI, and how to be confident in her ability as a new Mum

B Managing her health during and after pregnancy, life changes, partnership relationships and baby's needs.

C Managing psychosocial and mental health issues, understanding labour and giving birth.

PART B

QUESTION 1-6

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PART B

Text 5

Case study on Community acquired pneumonia

Mr Carbury's signs and symptoms suggested that he had community-acquired pneumonia (CAP). The definition of CAP varies in the literature but includes signs and symptoms related to the respiratory tract and the patient's general health (British Thoracic Society, 2009). Mr Carbury's baseline vital signs were recorded as part of a holistic assessment; his RR was at the upper end of normal – 20 breaths per minute (bpm) measured over a full 60 seconds, the depth of his breaths was shallow and the rhythm was regular, and his oxygen saturation (SpO2) was 93%. He was sitting on a chair but leaning slightly forwards, with his hands on his knees and reported chest discomfort on the right side. However, his chest movement appeared symmetrical and there was no paradoxical movement observed: in normal breathing the chest and abdomen should move in the same direction, while paradoxical breathing occurs when the chest and abdomen move in opposite directions.

Part B: QUESTIONS 1-6

Questions 1-6

5

Mt Carburys chest movements were

A

Equal and in sync with his stomach

B

Unequal and out of sync with his belly

C

Uneven and out of timing with his tummy

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PART B

QUESTION 1-6

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Part B

Part C

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Text 6

The distribution of discharge summaries

Once finalised, the Discharge Summary should be printed and given to the patient (and/or carer as appropriate) so they understand and can remember their follow-up instructions.

For patients referred to Outpatient Clinics for follow up, include the Clinic name and fax number in the CC section using the predetermined list contained within the EDS module of the Clinical Portal, and the referring doctor (if different to GP).

Electronic distribution of the Discharge Summary to the patient's nominated GP, any additional recipients noted in the CC section, and the Clinical Record Information System (CRIS), will occur automatically after "Finalisation" within the EDS module. The method of distribution of electronic discharge summaries is determined by the details listed for each GP/ Practice in ACTPAS, and the consent (to share information with the GP) recorded for the patient in ACTPAS at the time of discharge summary finalisation. If the patient has consented to participate in the Commonwealth Government *My Health Record* system, a copy of their Electronic Discharge Summary will also be distributed to their *My Health Record* at this time.

Handwritten Discharge Summaries received in the Clinical Record Service with the inpatient notes are manually faxed to the GP.

Part B: QUESTIONS 1-6

Questions 1-6

6

Which of the following lists correctly includes the various people or places that always receive a Discharge Summary?

A

Patient/carers, Outpatient clinic, patients nominated GP

B

Patient/carers, patient's My Health Record.

C

Patient/carers, patient's nominated GP

PART B

QUESTION 1-6

123456

Go to: Part BPart CText 1Text 2

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residents and improve their condition. According to a review published in the *Journal of Clinical Psychology*, such initiatives may have been enacted prematurely. Though it makes sense that contact with pets would have a positive impact, the truth is, from strictly a scientific standpoint, we don't really know for sure.

Human-animal interaction has been a subject of research for decades. Even so, the evidence is a wash, according to psychologist Molly Crossman, who conducted the review. Some studies find that interacting with pets has benefits, others do not. Another problem is the quality of such studies. For instance, those that look at animal-assisted therapy have only examined the short-term impact. Also, there isn't usually a control group to compare results to in these studies. So whether the impact is significant is in question and if so, whether or not the benefit is long-term. It is clear that human-animal interaction (HAI) has a small to medium impact in the near-term, but beyond that, we just don't know.

This kind of research may be tainted for another reason. In many of these settings, a trainer brings the animal in. But what effect is the trainer having on these outcomes? The positive benefits might derive not from interacting with the animal but its handler.

Owning a pet has also been determined to have health and psychological benefits. And although we can see those who have a dog or a cat at a young age are less likely to develop allergies, with the psychological advantages, since these were observational studies, the results are indeterminable, at least for now.

There are several theoretical reasons why interacting with friendly animals

9 A review published in the *Journal of Clinical Psychology* claims that

A Animals help people feel better

B Animals don't have any impact on the positive feelings of people

C The decision to incorporate animal therapy in various facilities may have been made hastily.

D Incorporating animal therapy in various facilities was a good decision.

10 What does the psychologist Molly Crossman mean when she says that the evidence is a wash?

A That the evidence is of no benefit to anyone

B That the evidence is weak

C That the evidence is bad

D That the evidence is convincing

11 Which of these things are not mentioned in the review?

Go to: **Part B** **Part C** **Text 1** **Text 2**

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Owning a pet has also been determined to have health and psychological benefits. And although we can see those who have a dog or a cat at a young age are less likely to develop allergies, with the psychological advantages, since these were observational studies, the results are indeterminable, at least for now.

There are several theoretical reasons why interacting with friendly animals on a regular basis may improve our condition. Animals and humans can share positive emotions, for instance. Say someone is terminally ill and suffering from chronic pain. They take part in animal-assisted therapy. Playing or cuddling with a cute dog or cat would improve their mood. Since mood has been proven to dampen or heighten pain, a better mood would hypothetically improve their condition. Yet, there is such a thing as the placebo effect.

11 Which of these things are not mentioned in the review?

A

The quality of the studies that have been conducted

B

The Lack of control groups

C

The long-term significance of human-animal interaction

D

The fact that only the short-term impact was considered

12 What does the writer claim about the physical and psychological benefits of owning a dog?

A

There is evidence that people owning a pet since early childhood are less likely to develop allergies.

B

There is evidence that there are many mental health benefits

C

The evidence is conclusive.

D

The evidence is dubious.

13 How can the terminally ill benefit from animal-assisted therapy?

Go to:

Part B

Part C

Text 1

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13

How can the terminally ill benefit from animal-assisted therapy?

A

They can play or cuddle with an animal

B

They may start feeling better

C

Their pain can be heightened

D

Their illness can be treated

14

Why is the author feeling skeptical in sixth Paragraph regarding the benefits from animal-assisted therapy?

A

Because there is a lack of solid evidence.

B

Because sometimes the belief of the patient in the positive outcome is beneficial

C

Because there are high chances that the mood can be hypothetically improved

D

Because there are only theoretical grounds for the claim.

PART C

QUESTION 7-22

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walk into our local pharmacy and pay a few dollars for a hundred pills we tend to think of those drugs as safe and practically benign.

One 2001 study shows that over 30 billion over-the-counter doses of NSAIDs (non-steroidal anti-inflammatory drugs) were purchased in the United States alone that year. Yet the safety of NSAIDs is of ongoing concern. One report states that roughly 110,000 people are injured or die due to acetaminophen (an analgesic in Tylenol, for one) each year. Given that there were 27 billion doses of that specific substance sold in 2009, that equals one complication in every quarter-million doses—not an epidemic, though nothing to gloss over either.

Since migraines are a chronic condition, Gottschalk is taking the long view on headaches. He recommends preventive options such as anti-seizure medications, low doses of tricyclic antidepressants, or higher doses of beta blockers. He's also a fan of nose sprays or injections given the tendency of certain pharmaceuticals to disrupt the digestive process. A common complaint of migraines is nausea, so feeding that intestinal fire means replacing one problem with another.

Looking into the future, Gottschalk is heading into research that helps the immune system address a protein triggered by migraines. The hopes are that an antibody infusion targeting this protein would stop common headaches before they start. Migraines have been shown to be progressively crippling with potential cardiovascular, cerebrovascular, and long-term neurologic effects. Experiencing even intermittent headaches, according to Gottschalk, could be the result of the same process that causes migraines.

17 Why does Gottschalk suggest treating every headache as a migraine?

- A In order to eradicate the headache.
- B To avoid buying potentially harmful medicine.
- C To achieve a durable effect.
- D To achieve a better effect.

18 What makes it so easy for people to pop pills?

- A Their availability at the chemist's.
- B The fact that they are very cheap.
- C The fact that they offer fast relief.
- D The fact that they are benign.

19 What is the author's opinion on NSAIDs?

- A They are deadly

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Text 2

Ingesting a nightly Advil or Tylenol is protocol for many. The general body aches, joint inflammation, and, especially, headaches that tag along on a stressful workday seem mitigated with a few hundred milligrams of pain relief. Of course, this only masks what could be a serious underlying condition.

We all have triggers for those sharp (or for some, dull) pains that attack the inside of our skull. Within moments a bottle is popped, two pills washed down in one gulp. The mistake we may be making, however, is confusing the trigger for the cause. The headache would most likely still have occurred, says Christopher Gottschalk. The trigger merely exacerbates the pain while giving your mind a supposed 'reason' to latch onto. Most of the things that people have thought of as triggers—stress, weather fronts, diet—when we have studied them, we've come up empty-handed every time. It's not that they cause headaches, but that when you're getting a headache, weather changes and perfumes feel worse.

As director of Yale Medicine's Headache and Facial Pain Center, Gottschalk has developed another approach: treat every headache as a migraine. This, he says, will offer more of an opportunity at long-term success in treating and even potentially eliminating the problem. This will take some work from a consumer standpoint, so accustomed we are to popping pills. Add to this that the cognitive distance between the freely perused aisles and the pharmacist's counter is not steep. When we can walk into our local pharmacy and pay a few dollars for a hundred pills we tend to think of those drugs as safe and practically benign.

Questions 15-22

15

Which of the following conditions can't be treated with Advil or Tylenol?

A

Body aches

B

Headaches

C

Joint inflammation

D

Stress

16

According to second paragraph, which of the following has been found to be a trigger for headaches?

A

perfume

B

changes in the weather

C

stress

D

none of the above

17

Why does Gottschalk suggest treating every headache as a migraine?

Go to: Part B Part C Text 1 Text 2

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19 What is the author's opinion on NSAIDs?

A They are deadly

B They are not completely safe

C They cause major injuries

D They cause complications.

20 What does the author imply when he says "nothing to gloss over either" in fourth Paragraph?

A That's nothing to show off about

B That's something that shouldn't be ignored or avoided

C That's nothing to be happy about.

D That's something that shouldn't be discussed

21 Which of these medications is considered to be safe for the stomach according to Gottschalk?

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walk into our local pharmacy and pay a few dollars for a hundred pills we tend to think of those drugs as safe and practically benign.

One 2001 study shows that over 30 billion over-the-counter doses of NSAIDs (non-steroidal anti-inflammatory drugs) were purchased in the United States alone that year. Yet the safety of NSAIDs is of ongoing concern. One report states that roughly 110,000 people are injured or die due to acetaminophen (an analgesic in Tylenol, for one) each year. Given that there were 27 billion doses of that specific substance sold in 2009, that equals one complication in every quarter-million doses—not an epidemic, though nothing to gloss over either.

Since migraines are a chronic condition, Gottschalk is taking the long view on headaches. He recommends preventive options such as anti-seizure medications, low doses of tricyclic antidepressants, or higher doses of beta blockers. He's also a fan of nose sprays or injections given the tendency of certain pharmaceuticals to disrupt the digestive process. A common complaint of migraines is nausea, so feeding that intestinal fire means replacing one problem with another.

Looking into the future, Gottschalk is heading into research that helps the immune system address a protein triggered by migraines. The hopes are that an antibody infusion targeting this protein would stop common headaches before they start. Migraines have been shown to be progressively crippling with potential cardiovascular, cerebrovascular, and long-term neurologic effects. Experiencing even intermittent headaches, according to Gottschalk, could be the result of the same process that causes migraines.

21

Which of these medications is considered to be safe for the stomach according to Gottschalk?

A

beta blockers

B

anti-seizure medications

C

injections

D

tricyclic antidepressants

22

What kind of therapy is Gottschalk hoping will help prevent migraines in the future?

A

Immunotherapy

B

Protein therapy

C

Neurological therapy

D

Meditation

PART C

QUESTION 7-22

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Part B

Part C

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PART B

Text 1

Device recall: Firmware Update to Address Cybersecurity Vulnerabilities Identified in Abbott's (formerly St. Jude Medical's) Implantable Cardiac Pacemakers: FDA Safety Communication for Health professionals

The FDA has reviewed information concerning potential cybersecurity vulnerabilities associated with St. Jude Medical's RF-enabled implantable cardiac pacemakers and has confirmed that these vulnerabilities, if exploited, could allow an unauthorized user to access a patient's device using commercially available equipment.

The FDA and Abbott do NOT recommend prophylactic removal and replacement of affected devices.

Discuss the risks and benefits of the cybersecurity vulnerabilities and associated firmware update with your patients at the next regularly scheduled visit. As part of this discussion, it is important to consider each patient's circumstances, such as pacemaker dependence, age of the device, and patient preference, and provide them with Abbott's Patient Guide.

For pacing dependent patients, consider performing the cybersecurity firmware update in a facility where temporary pacing and pacemaker generator can be readily provided.

Print or digitally store the programmed device settings and the diagnostic data in case of loss during the update.

Part B: QUESTIONS 1-6

Questions 1-6

1

What course of action must a healthcare professional take after reading this device recall notice?

A

The health professional should contact their patient so they can discuss issues raised in the recall notice

B

The health professional should discuss the pros and cons resulting from the firmware update at their next routine appointment

C

The health professional should discuss the possible effects of the firmware update and schedule replacing the device as soon as is convenient.

PART B

QUESTION 1-6

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Go to:

Part B

Part C

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The FDA has reviewed information concerning potential cybersecurity vulnerabilities associated with St. Jude Medical's RF-enabled implantable cardiac pacemakers and has confirmed that these vulnerabilities, if exploited, could allow an unauthorized user to access a patient's device using commercially available equipment.

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For pacing dependent patients, consider performing the cybersecurity firmware update in a facility where temporary pacing and pacemaker generator can be readily provided.

Print or digitally store the programmed device settings and the diagnostic data in case of loss during the update.

After the update, confirm that the device maintains its functionality, is not in backup mode, and that the programmed parameters have not changed.

The firmware update process is described in Abbott's Dear Doctor Letter issued on August 28, 2017.

Part B: QUESTIONS 1-6

Questions 1-6

1

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PART B

QUESTION 1-6

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Part B

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Text 2

Research into incidence of hospital treated Deliberate Self Harm

The incidence of hospital-treated DSH from institutional data was higher in females than males, both in Australia (148 vs 87per 100,000) and in New Zealand (86 vs 46per 100,000).

The incidence of hospital-treated DSH was highest among those aged 15–24 years in both

Australia and New Zealand predominantly due to higher rates in adolescents (15–19years). However, the peak of highest incidence of hospital-treated DSH in adolescents

and young adults in Australia is less marked when stratified by Aboriginal and Torres Strait

Islander status in Australia; rates of DSH among Aboriginal and Torres Strait Islander people aged 25–49years (males and females) are higher than for other Australians in the same age group. In Australia, the incidence of hospital-treated DSH among Aboriginal and Torres Strait Islander people was substantially higher than for the general population (292 vs 117per 100,000), males (259 vs 87 per 100,000) and females (325 vs 148per 100,000)

Part B: QUESTIONS 1-6

Questions 1-6

2

According to this research report which group of the Australian population are most likely to be treated in hospital for deliberate self-harm?

A

Women aged 15-19 from the Aboriginal and Torres Strait Island population

B

Women aged 15-24 from the Aboriginal and Torres Strait Island population

C

Women aged 25-49 from the Aboriginal and Torres Strait Island population.

PART B

QUESTION 1-6

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Go to:

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PART B

Text 4

Planning for subsequent antenatal visits

At all visits, opportunities should be provided for the woman to share her expectations and experiences as well as discuss any issues and/or concerns that may have arisen since her last visit, including psychosocial support and mental health issues. Women should also be offered information on aspects of health in pregnancy and early parenthood (e.g. nutrition, alcohol, smoking, symptom relief if conditions common in pregnancy are being experienced, breastfeeding, reducing the risk of sudden and unexpected death in infancy [SUDI]). A woman's confidence in her ability to labour, give birth and look after her new baby should be supported throughout antenatal care and antenatal education should also support her in preparing for changes to her life and her relationship with her partner and understanding the physical and emotional needs of the baby. The woman's needs should dictate the type of information and support provided (e.g. while many women will benefit from written information, other forms of information such as audio or video are sometimes more suitable). The woman should also direct the type of issues and questions discussed.

Part B: QUESTIONS 1-6

Questions 1-6

4 What are the main topics women should be given information on during their ante natal period?

- A Managing her expectations, knowing which babies are at risk of SUDI, and how to be confident in her ability as a new Mum
- B Managing her health during and after pregnancy, life changes, partnership relationships and baby's needs.
- C Managing psychosocial and mental health issues, understanding labour and giving birth.

PART B

QUESTION 1-6

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PART B

Text 5

Case study on Community acquired pneumonia

Mr Carbury's signs and symptoms suggested that he had community-acquired pneumonia (CAP). The definition of CAP varies in the literature but includes signs and symptoms related to the respiratory tract and the patient's general health (British Thoracic Society, 2009). Mr Carbury's baseline vital signs were recorded as part of a holistic assessment; his RR was at the upper end of normal – 20 breaths per minute (bpm) measured over a full 60 seconds, the depth of his breaths was shallow and the rhythm was regular, and his oxygen saturation (SpO2) was 93%. He was sitting on a chair but leaning slightly forwards, with his hands on his knees and reported chest discomfort on the right side. However, his chest movement appeared symmetrical and there was no paradoxical movement observed: in normal breathing the chest and abdomen should move in the same direction, while paradoxical breathing occurs when the chest and abdomen move in opposite directions.

Part B: QUESTIONS 1-6

Questions 1-6

5 Mt Carburys chest movements were

A Equal and in sync with his stomach

B Unequal and out of sync with his belly

C Uneven and out of timing with his tummy

PART B

QUESTION 1-6

123456

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PART B

Text 6

The distribution of discharge summaries

Once finalised, the Discharge Summary should be printed and given to the patient (and/or carer as appropriate) so they understand and can remember their follow-up instructions.

For patients referred to Outpatient Clinics for follow up, include the Clinic name and fax number in the CC section using the predetermined list contained within the EDS module of the Clinical Portal, and the referring doctor (if different to GP).

Electronic distribution of the Discharge Summary to the patient's nominated GP, any additional recipients noted in the CC section, and the Clinical Record Information System (CRIS), will occur automatically after "Finalisation" within the EDS module. The method of distribution of electronic discharge summaries is determined by the details listed for each GP/ Practice in ACTPAS, and the consent (to share information with the GP) recorded for the patient in ACTPAS at the time of discharge summary finalisation. If the patient has consented to participate in the Commonwealth Government *My Health Record* system, a copy of their Electronic Discharge Summary will also be distributed to their *My Health Record* at this time.

Handwritten Discharge Summaries received in the Clinical Record Service with the inpatient notes are manually faxed to the GP.

Part B: QUESTIONS 1-6

Questions 1-6

6

Which of the following lists correctly includes the various people or places that always receive a Discharge Summary?

A

Patient/carers, Outpatient clinic, patients nominated GP

B

Patient/carers, patient's My Health Record.

C

Patient/carers, patient's nominated GP

PART B

QUESTION 1-6

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as more and more hospitals, nursing homes, universities, prisons, and other such facilities, are using animal-assisted therapy as a way to calm residents and improve their condition. According to a review published in the *Journal of Clinical Psychology*, such initiatives may have been enacted prematurely. Though it makes sense that contact with pets would have a positive impact, the truth is, from strictly a scientific standpoint, we don't really know for sure.

Human-animal interaction has been a subject of research for decades. Even so, the evidence is a wash, according to psychologist Molly Crossman, who conducted the review. Some studies find that interacting with pets has benefits, others do not. Another problem is the quality of such studies. For instance, those that look at animal-assisted therapy have only examined the short-term impact. Also, there isn't usually a control group to compare results to in these studies. So whether the impact is significant is in question and if so, whether or not the benefit is long-term. It is clear that human-animal interaction (HAI) has a small to medium impact in the near-term, but beyond that, we just don't know.

This kind of research may be tainted for another reason. In many of these settings, a trainer brings the animal in. But what effect is the trainer having on these outcomes? The positive benefits might derive not from interacting with the animal but its handler.

Owning a pet has also been determined to have health and psychological benefits. And although we can see those who have a dog or a cat at a young age are less likely to develop allergies, with the psychological advantages, since these were observational studies, the results are indeterminable, at least for now.

9 A review published in the *Journal of Clinical Psychology* claims that

A Animals help people feel better

B Animals don't have any impact on the positive feelings of people

C The decision to incorporate animal therapy in various facilities may have been made hastily.

D Incorporating animal therapy in various facilities was a good decision.

10 What does the psychologist Molly Crossman mean when she says that the evidence is a wash?

A That the evidence is of no benefit to anyone

B That the evidence is weak

C That the evidence is bad

D That the evidence is convincing

11 Which of these things are not mentioned in the review?

Go to: Part B Part C Text 1 Text 2

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This kind of research may be tainted for another reason. In many of these settings, a trainer brings the animal in. But what effect is the trainer having on these outcomes? The positive benefits might derive not from interacting with the animal but its handler.

Owning a pet has also been determined to have health and psychological benefits. And although we can see those who have a dog or a cat at a young age are less likely to develop allergies, with the psychological advantages, since these were observational studies, the results are indeterminable, at least for now.

There are several theoretical reasons why interacting with friendly animals on a regular basis may improve our condition. Animals and humans can share positive emotions, for instance. Say someone is terminally ill and suffering from chronic pain. They take part in animal-assisted therapy. Playing or cuddling with a cute dog or cat would improve their mood. Since mood has been proven to dampen or heighten pain, a better mood would hypothetically improve their condition. Yet, there is such a thing as the placebo effect.

11

Which of these things are not mentioned in the review?

A

The quality of the studies that have been conducted

B

The Lack of control groups

C

The long-term significance of human-animal interaction

D

The fact that only the short-term impact was considered

12

What does the writer claim about the physical and psychological benefits of owning a dog?

A

There is evidence that people owning a pet since early childhood are less likely to develop allergies.

B

There is evidence that there are many mental health benefits

C

The evidence is conclusive.

D

The evidence is dubious.

13

How can the terminally ill benefit from animal-assisted therapy?

Go to:

Part B

Part C

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13

How can the terminally ill benefit from animal-assisted therapy?

A

They can play or cuddle with an animal

B

They may start feeling better

C

Their pain can be heightened

D

Their illness can be treated

14

Why is the author feeling skeptical in sixth Paragraph regarding the benefits from animal-assisted therapy?

A

Because there is a lack of solid evidence.

B

Because sometimes the belief of the patient in the positive outcome is beneficial

C

Because there are high chances that the mood can be hypothetically improved

D

Because there are only theoretical grounds for the claim.

PART C

QUESTION 7-22

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Part B

Part C

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PART C

Text 2

Ingesting a nightly Advil or Tylenol is protocol for many. The general body aches, joint inflammation, and, especially, headaches that tag along on a stressful workday seem mitigated with a few hundred milligrams of pain relief. Of course, this only masks what could be a serious underlying condition.

We all have triggers for those sharp (or for some, dull) pains that attack the inside of our skull. Within moments a bottle is popped, two pills washed down in one gulp. The mistake we may be making, however, is confusing the trigger for the cause. The headache would most likely still have occurred, says Christopher Gottschalk. The trigger merely exacerbates the pain while giving your mind a supposed 'reason' to latch onto. Most of the things that people have thought of as triggers—stress, weather fronts, diet—when we have studied them, we've come up empty-handed every time. It's not that they cause headaches, but that when you're getting a headache, weather changes and perfumes feel worse.

As director of Yale Medicine's Headache and Facial Pain Center, Gottschalk has developed another approach: treat every headache as a migraine. This, he says, will offer more of an opportunity at long-term success in treating and even potentially eliminating the problem. This will take some work from a consumer standpoint, so accustomed we are to popping pills. Add to this that the cognitive distance between the freely perused aisles and the pharmacist's counter is not steep. When we can

Part C: QUESTIONS 7-22

Questions 15-22

15 Which of the following conditions can't be treated with Advil or Tylenol?

A Body aches

B Headaches

C Joint inflammation

D Stress

16 According to second paragraph, which of the following has been found to be a trigger for headaches?

A perfume

B changes in the weather

C stress

D none of the above

Go to: **Part B** **Part C** **Text 1** **Text 2**

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As director of Yale Medicine's Headache and Facial Pain Center, Gottschalk has developed another approach: treat every headache as a migraine. This, he says, will offer more of an opportunity at long-term success in treating and even potentially eliminating the problem. This will take some work from a consumer standpoint, so accustomed we are to popping pills. Add to this that the cognitive distance between the freely perused aisles and the pharmacist's counter is not steep. When we can walk into our local pharmacy and pay a few dollars for a hundred pills we tend to think of those drugs as safe and practically benign.

One 2001 study shows that over 30 billion over-the-counter doses of NSAIDs (non-steroidal anti-inflammatory drugs) were purchased in the United States alone that year. Yet the safety of NSAIDs is of ongoing concern. One report states that roughly 110,000 people are injured or die due to acetaminophen (an analgesic in Tylenol, for one) each year. Given that there were 27 billion doses of that specific substance sold in 2009, that equals one complication in every quarter-million doses—not an epidemic, though nothing to gloss over either.

Since migraines are a chronic condition, Gottschalk is taking the long view on headaches. He recommends preventive options such as anti-seizure medications, low doses of tricyclic antidepressants, or higher doses of beta blockers. He's also a fan of nose sprays or injections given the tendency of certain pharmaceuticals to disrupt the digestive process. A common complaint of migraines is nausea, so feeding that intestinal fire means replacing one problem with another.

Looking into the future, Gottschalk is heading into research that helps the immune system address a protein triggered by migraines. The hopes are that an antibody infusion targeting this protein would stop common headaches before they start.

17 Why does Gottschalk suggest treating every headache as a migraine?

- A In order to eradicate the headache.
- B To avoid buying potentially harmful medicine.
- C To achieve a durable effect.
- D To achieve a better effect.

18 What makes it so easy for people to pop pills?

- A Their availability at the chemist's.
- B The fact that they are very cheap.
- C The fact that they offer fast relief.
- D The fact that they are benign.

19 What is the author's opinion on NSAIDs?

- A They are deadly

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One 2001 study shows that over 30 billion over-the-counter doses of NSAIDs (non-steroidal anti-inflammatory drugs) were purchased in the United States alone that year. Yet the safety of NSAIDs is of ongoing concern. One report states that roughly 110,000 people are injured or die due to acetaminophen (an analgesic in Tylenol, for one) each year. Given that there were 27 billion doses of that specific substance sold in 2009, that equals one complication in every quarter-million doses—not an epidemic, though nothing to gloss over either.

Since migraines are a chronic condition, Gottschalk is taking the long view on headaches. He recommends preventive options such as anti-seizure medications, low doses of tricyclic antidepressants, or higher doses of beta blockers. He's also a fan of nose sprays or injections given the tendency of certain pharmaceuticals to disrupt the digestive process. A common complaint of migraines is nausea, so feeding that intestinal fire means replacing one problem with another.

Looking into the future, Gottschalk is heading into research that helps the immune system address a protein triggered by migraines. The hopes are that an antibody infusion targeting this protein would stop common headaches before they start. Migraines have been shown to be progressively crippling with potential cardiovascular, cerebrovascular, and long-term neurologic effects. Experiencing even intermittent headaches, according to Gottschalk, could be the result of the same process that causes migraines.

19 What is the author's opinion on NSAIDs?

A They are deadly

B They are not completely safe

C They cause major injuries

D They cause complications.

20 What does the author imply when he says "nothing to gloss over either" in fourth Paragraph?

A That's nothing to show off about

B That's something that shouldn't be ignored or avoided

C That's nothing to be happy about.

D That's something that shouldn't be discussed

21 Which of these medications is considered to be safe for the stomach according to Gottschalk?

Go to: **Part B** **Part C** **Text 1** **Text 2**

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walk into our local pharmacy and pay a few dollars for a hundred pills we tend to think of those drugs as safe and practically benign.

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21 Which of these medications is considered to be safe for the stomach according to Gottschalk?

A beta blockers

B anti-seizure medications

C injections

D tricyclic antidepressants

22 What kind of therapy is Gottschalk hoping will help prevent migraines in the future?

A Immunotherapy

B Protein therapy

C Neurological therapy

D Meditation

PART C

QUESTION 7-22

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Go to: **Part B** **Part C** **Text 1** **Text 2**

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